



Patient Information	Specimen Information	Client Information
GOPALAKRISHNAN, SRIKRISHNA DOB: 08/11/1978 AGE: 47 Gender: M Fasting: Y Phone: 408.416.6993 Patient ID: 0B367884 Health ID: 8573038557169760	Specimen: SZ716806X Requisition: 0179834 Lab Ref #: D402D598B4144427 Collected: 02/25/2026 / 09:24 PST Received: 02/25/2026 / 23:51 PST Reported: 03/06/2026 / 13:55 PST	Client #: 73929412 MAIL992 DAMASCO, LEO JUNCTION 440 N BARRANCA AVE # 3811 COVINA, CA 91723-1722

COMMENTS: PATIENT MUST BE FASTING
FASTING: YES

Test Name	In Range	Out Of Range	Reference Range	Lab
IRON, TIBC AND FERRITIN PANEL				
IRON AND TOTAL IRON				UL
BINDING CAPACITY				
IRON, TOTAL	144		50-180 mcg/dL	
IRON BINDING CAPACITY	335		250-425 mcg/dL (calc)	
% SATURATION	43		20-48 % (calc)	
FERRITIN	120		38-380 ng/mL	UL
THYROID PANEL WITH TSH				
THYROID PANEL				UL
T3 UPTAKE	35		22-35 %	
T4 (THYROXINE), TOTAL	5.9		4.9-10.5 mcg/dL	
FREE T4 INDEX (T7)	2.1		1.4-3.8	
TSH	3.17		0.40-4.50 mIU/L	UL
TESTOSTERONE, FREE, BIOAVAILABLE AND TOTAL, MS				
ALBUMIN	4.6		3.6-5.1 g/dL	EZ
SEX HORMONE BINDING				EZ
GLOBULIN	20		10-50 nmol/L	
TESTOSTERONE, TOTAL, MS	419		250-1100 ng/dL	EZ

For additional information, please refer to
<http://education.questdiagnostics.com/faq/TotalTestosteroneL>
CMSMS (This link is being provided for
informational/educational purposes only.)

This test was developed and its analytical performance
characteristics have been determined by Quest Diagnostics.
It has not been cleared or approved by the FDA. This assay
has been validated pursuant to the CLIA regulations and is
used for clinical purposes.

TESTOSTERONE, FREE AND BIOAVAILABLE				EZ
TESTOSTERONE, FREE	80.1		46.0-224.0 pg/mL	

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characteristics have been determined by Quest Diagnostics.
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TESTOSTERONE, BIOAVAILABLE	168.2		110.0-575.0 ng/dL	
LIPOPROTEIN (a)		76 H	nmol/L	EN
		Reference Range	<75	

Risk:
Optimal <75
Moderate 75-125
High >125

Cardiovascular event risk category
cut points (optimal, moderate, high)
are based on Tsimika S. JACC



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LIPID PANEL WITH RATIOS

CHOLESTEROL, TOTAL

HDL CHOLESTEROL

TRIGLYCERIDES

LDL-CHOLESTEROL

Reference range: <100

Desirable range <100 mg/dL for primary prevention;
<70 mg/dL for patients with CHD or diabetic patients
with > or = 2 CHD risk factors.

LDL-C is now calculated using the Martin-Hopkins
calculation, which is a validated novel method providing
better accuracy than the Friedewald equation in the
estimation of LDL-C.

Martin SS et al. JAMA. 2013;310(19): 2061-2068
(<http://education.QuestDiagnostics.com/faq/FAQ164>)

CHOL/HDLRATIO

LDL/HDL RATIO

4.7

3.3

Below Average Risk:

Average Risk:

Moderate Risk:

High Risk:

<5.0 (calc)

(calc)

<2.28

2.29-4.90

4.91-7.12

>7.13

UL

UL

UL

UL

UL

UL

NON HDL CHOLESTEROL
176 H

<130 mg/dL (calc)

UL

For patients with diabetes plus 1 major ASCVD risk
factor, treating to a non-HDL-C goal of <100 mg/dL
(LDL-C of <70 mg/dL) is considered a therapeutic
option.

HS CRP

0.8

mg/L

UL

Reference Range

Optimal <1.0

Jellinger PS et al. Endocr Pract.2017;23(Suppl 2):1-87.

For ages >17 Years:

hs-CRP mg/L Risk According to AHA/CDC Guidelines

<1.0 Lower relative cardiovascular risk.

1.0-3.0 Average relative cardiovascular risk.

3.1-10.0 Higher relative cardiovascular risk.

Consider retesting in 1 to 2 weeks to

exclude a benign transient elevation

in the baseline CRP value secondary

to infection or inflammation.

>10.0 Persistent elevation, upon retesting,

may be associated with infection and

inflammation.

Pearson TA, Mensah GA, Alexander RW, et al. Markers
of inflammation and cardiovascular disease:
application to clinical and public health practice:
A statement for healthcare professionals from the
Centers for Disease Control and Prevention and the
American Heart Association. Circulation 2003; 107(3):
499-511.

APOLIPOPROTEIN B
109 H

mg/dL

EN

Reference Range <90



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Risk Category:

Optimal	<90
Moderate	90-129
High	> or = 130

A desirable treatment target may be <80 mg/dL or lower depending on the risk category of the patient including patients on lipid lowering therapies, patients with ASCVD, diabetes with >1 risk factors, Stage 3 or greater CKD with albuminuria, or heterozygous familial hypercholesterolemia. ApoB relative risk category cut points are based on AACE/ACE and ACC/AHA recommendations (Grundy SM, et al. 2019. doi:10.1016/j.jacc.2018.11.002; Handelsman Y, et al. 2020. doi:10.4158/CS-2020-0490).

COMPREHENSIVE METABOLIC

UL

PANEL

GLUCOSE

103 H

65-99 mg/dL

Fasting reference interval

For someone without known diabetes, a glucose value between 100 and 125 mg/dL is consistent with prediabetes and should be confirmed with a follow-up test.

UREA NITROGEN (BUN)	15	7-25 mg/dL
CREATININE	1.04	0.60-1.29 mg/dL
EGFR	89	> OR = 60 mL/min/1.73m2
BUN/CREATININE RATIO	14	6-22 (calc)
SODIUM	139	135-146 mmol/L
POTASSIUM	3.9	3.5-5.3 mmol/L
CHLORIDE	104	98-110 mmol/L
CARBON DIOXIDE	27	20-32 mmol/L
CALCIUM	9.5	8.6-10.3 mg/dL
PROTEIN, TOTAL	7.0	6.1-8.1 g/dL
ALBUMIN	4.6	3.6-5.1 g/dL
GLOBULIN	2.4	1.9-3.7 g/dL (calc)
ALBUMIN/GLOBULIN RATIO	1.9	1.0-2.5 (calc)
BILIRUBIN, TOTAL	0.8	0.2-1.2 mg/dL
ALKALINE PHOSPHATASE	41	36-130 U/L
AST	21	10-40 U/L
ALT	25	9-46 U/L

HEMOGLOBIN A1c WITH eAG

HEMOGLOBIN A1c

5.8 H

<5.7 %

UL

For someone without known diabetes, a hemoglobin A1c value between 5.7% and 6.4% is consistent with prediabetes and should be confirmed with a follow-up test.

For someone with known diabetes, a value <7%



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indicates that their diabetes is well controlled. A1c targets should be individualized based on duration of diabetes, age, comorbid conditions, and other considerations.

This assay result is consistent with an increased risk of diabetes.

Currently, no consensus exists regarding use of hemoglobin A1c for diagnosis of diabetes for children.

eAG (mg/dL)	120	mg/dL	
eAG (mmol/L)	6.6	mmol/L	
URIC ACID	6.5	4.0-8.0 mg/dL	UL
Therapeutic target for gout patients: <6.0 mg/dL			
BILIRUBIN, FRACTIONATED			UL
BILIRUBIN, TOTAL	0.8	0.2-1.2 mg/dL	
BILIRUBIN, DIRECT	0.1	< OR = 0.2 mg/dL	
BILIRUBIN, INDIRECT	0.7	0.2-1.2 mg/dL (calc)	
GGT	29	3-95 U/L	UL
VITAMIN C			AMD
VITAMIN C	0.7	0.2-2.1 mg/dL	

Vitamin C results can be affected by poor temperature control during sample transport. If test results are below the normal range and do not correlate with clinical findings, recollection may be appropriate and specimen temperature at -70 C should be ensured. To shorten the time between collection and transport, please collect sample after 12 noon.

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VITAMIN K	450	130-1500 pg/mL	AMD
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CBC (INCLUDES DIFF/PLT)			UL
WHITE BLOOD CELL COUNT	4.8	3.8-10.8 Thousand/uL	
RED BLOOD CELL COUNT	5.01	4.20-5.80 Million/uL	
HEMOGLOBIN	15.0	13.2-17.1 g/dL	
HEMATOCRIT	44.6	39.4-51.1 %	
MCV	89.0	81.4-101.7 fL	



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Test Name	In Range	Out Of Range	Reference Range	Lab
MCH	29.9		27.0-33.0 pg	
MCHC	33.6		31.6-35.4 g/dL	
RDW	12.2		11.0-15.0 %	
PLATELET COUNT	262		140-400 Thousand/uL	
MPV	9.7		7.5-12.5 fL	
ABSOLUTE NEUTROPHILS	2515		1500-7800 cells/uL	
ABSOLUTE LYMPHOCYTES	1690		850-3900 cells/uL	
ABSOLUTE MONOCYTES	394		200-950 cells/uL	
ABSOLUTE EOSINOPHILS	163		15-500 cells/uL	
ABSOLUTE BASOPHILS	38		0-200 cells/uL	
NEUTROPHILS	52.4		%	
LYMPHOCYTES	35.2		%	
MONOCYTES	8.2		%	
EOSINOPHILS	3.4		%	
BASOPHILS	0.8		%	
VITAMIN B12/FOLATE, SERUM PANEL				UL
VITAMIN B12	455		200-1100 pg/mL	
FOLATE, SERUM	12.5		ng/mL	
			Reference Range	
			Low: <3.4	
			Borderline: 3.4-5.4	
			Normal: >5.4	
DHEA SULFATE	242		61-442 mcg/dL	EN
CORTISOL, TOTAL	8.7		mcg/dL	UL
			Reference Range: For 8 a.m.(7-9 a.m.) Specimen: 4.0-22.0	
			Reference Range: For 4 p.m.(3-5 p.m.) Specimen: 3.0-17.0	
			* Please interpret above results accordingly *	
ESTRADIOL	<30		< OR = 39 pg/mL	UL
			Reference range established on post-pubertal patient population. No pre-pubertal reference range established using this assay. For any patients for whom low Estradiol levels are anticipated (e.g. males, pre-pubertal children and hypogonadal/post-menopausal females), the Quest Diagnostics Nichols Institute Estradiol, Ultrasensitive, LCMSMS assay is recommended (order code 30289).	
			Please note: patients being treated with the drug fulvestrant (Faslodex(R)) have demonstrated significant interference in immunoassay methods for estradiol measurement. The cross reactivity could lead to falsely elevated estradiol test results leading to an inappropriate clinical assessment of estrogen status. Quest Diagnostics order code 30289-Estradiol, Ultrasensitive LC/MS/MS demonstrates negligible cross reactivity with fulvestrant.	
VITAMIN E (TOCOPHEROL)				Z3E
VITAMIN E, ALPHA TOCOPHEROL	11.2		5.7-19.9 mg/L	
			Levels of alpha-tocopherol <5 mg/L are consistent with Vitamin E deficiency in adults.	
VITAMIN E, BETA GAMMA TOCOPHEROL	1.1		<4.4 mg/L	
			(Note)	



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Vitamin supplementation within 24 hours prior to blood draw may affect the accuracy of results.

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MDF
med fusion
2501 South State Highway 121, Suite 1100
Lewisville TX 75067
972-966-7300
Ithiel James L. Frame, MD, PhD

MAGNESIUM, RBC	5.4	4.0-6.4 mg/dL	Z3E
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(Note)
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MDF
med fusion
2501 South State Highway 121, Suite 1100
Lewisville TX 75067
972-966-7300
Ithiel James L. Frame, MD, PhD

SELENIUM	144	63-160 mcg/L	Z3E
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(Note)
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MDF
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2501 South State Highway 121, Suite 1100
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972-966-7300
Ithiel James L. Frame, MD, PhD



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Endocrinology

Test Name	Result	Reference Range	Lab
VITAMIN D,25-OH,TOTAL,IA	86	30-100 ng/mL	UL
Vitamin D Status 25-OH Vitamin D: Deficiency: <20 ng/mL Insufficiency: 20 - 29 ng/mL Optimal: > or = 30 ng/mL For 25-OH Vitamin D testing on patients on D2-supplementation and patients for whom quantitation of D2 and D3 fractions is required, the QuestAssureD(TM) 25-OH VIT D, (D2,D3), LC/MS/MS is recommended: order code 92888 (patients >2yrs).			
For additional information, please refer to http://education.QuestDiagnostics.com/faq/FAQ199 (This link is being provided for informational/ educational purposes only.)			
Physician Comments:			



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Cardio IQ®

Test Name	Current		Risk/Reference Interval			Units	Historical
	Result & Risk		Optimal	Moderate	High		
	Optimal	Non-Optimal					Result & Risk
LIPOPROTEIN FRACTIONATION NMR							
LDL P		2143	<935	935-1816	>1816	nmol/L	
SMALL LDL P		672	<467	467-820	>820	nmol/L	
LDL SIZE	21.3		>20.5	NA	<20.6	nm	
HDL P	35.4		>32.8	29.2-32.8	<29.2	umol/L	
LARGE HDL P		<3.0	>7.2	5.3-7.2	<5.3	umol/L	
HDL SIZE		8.2	>9.0	8.7-9.0	<8.7	nm	
LARGE VLDL P	3.3		<3.7	3.7-6.1	>6.1	nmol/L	
VLDL SIZE	45.4		<47.1	47.1-49.0	>49.0	nm	
INFLAMMATION							
ADMA (Asymmetric dimethylarginine)		102	<100	100-123	>123	ng/mL	
SDMA (Symmetric dimethylarginine)	81			73-135		ng/mL	

For details on reference ranges please refer to the reference range/comment section of the report.

Medical Information For Healthcare Providers: If you have questions about any of the tests in our Cardio IQ offering, please call Client Services at our Quest Diagnostics-Cleveland HeartLab Cardiometabolic Center of Excellence. They can be reached at 866.358.9828, option 1 to arrange a consult with our clinical education team.



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Reference Range/Comments				
Analyte Name	In Range	Out Range	Reference Range	Lab
ADMA		102	<100 ng/mL	Z4M
This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics Cardiometabolic Center of Excellence at Cleveland HeartLab. It has not been cleared or approved by the U.S. Food and Drug Administration. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes. Elevated ADMA levels are associated with significant subclinical atherosclerosis while elevated SDMA levels are associated with kidney function and strongly correlate with reduced eGFR. Available prospective studies suggest an increased risk of cardiovascular disease with higher ADMA concentrations (1). Based on an internal reference range study using 180 'apparently healthy,' non-smoking donors, CHL has defined the following cut-offs for ADMA: A cut-off of <100 ng/mL defines an 'apparently healthy' population at optimal relative risk for a cardiovascular event, 100-123 ng/mL defines a population at moderate relative risk for a cardiovascular event, and >123 ng/mL defines a high relative risk population. (Reference: 1-Willeit P. et al. J Am Heart Assoc. 2015; 4: e001833).				
HDL SIZE		8.2	>9.0 nm	Z4M
Relative risk: Optimal >9.0; Moderate 8.7-9.0; High <8.7 nm. Reference range is 8.3-10.5 nm.				
LARGE HDL P		<3.0	>7.2 umol/L	Z4M
Relative risk: Optimal >7.2; Moderate 5.3-7.2; High <5.3 umol/L. Reference range is >3.5 umol/L.				
LDL P		2143	<935 nmol/L	Z4M
This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics Cardiometabolic Center of Excellence at Cleveland HeartLab. It has not been cleared or approved by the U.S. Food and Drug Administration. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes. Relative risk: Optimal <935; Moderate 935-1816; High >1816 nmol/L. Reference range is 592-2404 nmol/L.				
SMALL LDL P		672	<467 nmol/L	Z4M
Relative risk: Optimal <467; Moderate 467-820; High >820 nmol/L. Reference range is <1408 nmol/L.				
HDL P	35.4		>32.8 umol/L	Z4M
Relative risk: Optimal >32.8; Moderate 29.2-32.8; High <29.2 umol/L. Reference range is 21.1-43.4 umol/L.				
LARGE VLDL P	3.3		<3.7 nmol/L	Z4M
Relative risk: Optimal <3.7; Moderate 3.7-6.1; High >6.1 nmol/L. Reference range is <16.0 nmol/L.				
LDL SIZE	21.3		>20.5 nm	Z4M
Relative risk: Optimal >20.5; High <20.6 nm. Reference range is 20.0-22.3 nm.				
SDMA	81		73-135 ng/mL	Z4M
VLDL SIZE	45.4		<47.1 nm	Z4M
Relative risk: Optimal <47.1; Moderate 47.1-49.0; High >49.0 nm. Reference range is 41.1-61.7 nm.				

PERFORMING SITE:

AMD QUEST DIAGNOSTICS/NICHOLS CHANTILLY, 14225 NEWBROOK DRIVE, CHANTILLY, VA 20151-2228 Laboratory Director: PATRICK W. MASON,MD,PHD, CLIA: 49D0221801
EN QUEST DIAGNOSTICS-WEST HILLS, 8401 FALLBROOK AVENUE, WEST HILLS, CA 91304-3226 Laboratory Director: THOMAS MCDONALD, MD, CLIA: 05D0642827
EZ QUEST DIAGNOSTICS/NICHOLS SJC, 33608 ORTEGA HWY, SAN JUAN CAPISTRANO, CA 92675-2042 Laboratory Director: IRINA MARAMICA,MD,PHD,MBA, CLIA: 05D0643352
UL QUEST DIAGNOSTICS SACRAMENTO, 3714 NORTHGATE BLVD, SACRAMENTO, CA 95834-1617 Laboratory Director: LORNE L. HOLLAND, MD, CLIA: 05D0644209
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Z4M CLEVELAND HEARTLAB INC, 6701 CARNEGIE AVENUE SUITE 500, CLEVELAND, OH 44103-4623 Laboratory Director: M. QASIM ANSARI, MD , CLIA: 36D1032987